

# Request for Information 70RWMD26RFIP00005

Department of Homeland Security | Office of Health Security | Occupational/Employee Health Electronic Health Record & Medical Information System

\* Required

## Instructions

Provide detailed responses to the questions below, tailored to the DHS Occupational Health EHR / Management Information System context described in the RFI and draft Statement of Objectives. DHS recognizes that this is an RFI and that vendors are **not reimbursed** for the time and resources required to respond. To help balance burden and usefulness:

- You **do not need to answer every question**
- Please **prioritize the questions that best showcase your core capabilities and deployment approach** and/or where you believe additional clarity from DHS would be most helpful.
- As a general guide, DHS is most interested in your responses to the questions in **Sections 1-5**, and invites you to answer **at least 20 questions total** across these sections that you feel best represent your solution.
- For any question you choose not to answer, you may leave it blank or indicate “Not Addressed.”

If a question clearly does not apply to your solution, please state “**Not Applicable (N/A)**” and briefly explain why.

Where relevant, describe how you would demonstrate specific capabilities during a live, in-person one-on-one capability demonstration during Industry Day.

## Section 1 – Corporate Information and Qualifications

1. Legal Business Name \*

2. Company Website \*

3. Point of Contact's Name, E-mail, and Phone Number \*

4. Is this company registered in the System for Award Management (SAM)? \*

☐ Yes

☐ No

5. If you answer Yes to Question 4, provide the company's Unique Entity Identifier (UEI) Number

6. Provide the associated NAICS code under which this company conducts business.

7. **Company Profile** Provide an overview of your company, including years in business, headquarters location, number of employees, and primary lines of business related to occupational/employee health or health IT.

Please enter at most 2000 characters

8. **Company Profile** Describe your financial stability and capacity to support a multi-year implementation serving approximately 200,000 employees across a geographically distributed federal enterprise.

Please enter at most 2000 characters

9. **Company Profile** Indicate the Business Size (Large or Small) and Socio-Economic Status for this company. \*

- ☐ Small Business
- ☐ Small Disadvantaged Business
- ☐ HUBZone Business
- ☐ Service Disabled Veteran Owned Small Business
- ☐ Veteran Owned Small Business
- ☐ Woman-Owned Small Business
- ☐ Large Business
- ☐ Other

10. **Relevant Experience** Describe your experience implementing occupational/employee health EHR or closely related systems for federal agencies or large employers (10,000+ employees). Include:

- Number of implementations;
- Types of organizations (e.g., federal, state/local, private);
- Approximate user counts, locations, and populations served

Please enter at most 2000 characters

11. **Relevant Experience** Describe your experience providing a comprehensive electronic health record (EHR) capability for occupational health providers, including medical documentation, prescribing, laboratory ordering and results, procedures, patient plans, and other core clinical functions needed to document and manage encounters such as periodic exams and post-exposure exams performed by providers.

Please enter at most 2000 characters

12. **Relevant Experience** Describe any experience supporting occupational health for law enforcement, national security, military, or public safety organizations with medical readiness and fitness-for-duty requirements.

Please enter at most 2000 characters

13. **Relevant Experience** Provide at least three (3) references and no more than five (5) for implementations of similar scope and complexity (federal references preferred). Include organization name, contact information, project scope, and implementation timeframe.

Please enter at most 2000 characters

14. **Relevant Experience** If proposing as a team (e.g., product vendor plus systems integrator), describe the teaming arrangement, roles and responsibilities, and any prior joint engagements.

Please enter at most 2000 characters

## Section 2 – Solution Overview and Architecture

15. **Product Information** Identify the product(s) you are proposing as the DHS Occupational Health EHR / Management Information System (single integrated platform vs. suite of products). Include current version and release date.

Please enter at most 2000 characters

16. **Product Information** For each product identified in your response to question 15, describe whether the solution is marketed primarily as:

- An occupational/employee health EHR system; or
- A general EHR with occupational modules; or
- Another type of platform (e.g., case management, workflow, CRM).

Please enter at most 2000 characters

17. **Architecture and Hosting** Describe your system architecture (cloud-native, cloud-hosted, on-premises, hybrid) and multi-tenancy model.

Please enter at most 2000 characters

18. **Architecture and Hosting** Identify supported cloud service providers and deployment models suitable for DHS (e.g., dedicated tenant, GovCloud).

Please enter at most 2000 characters

19. **Architecture and Hosting.** Explain how your architecture would support:

- Component-level configuration and logical separation;
- Shared enterprise services such as a unified analytics/reporting layer;
- A layered approach consisting of:

- o A core clinical/occupational EHR layer (encounters, documentation, orders, results);
- o An occupational/exposure data layer that maintains longitudinal, exposure-centric employee records (jobs, locations, SEGs, hazards, incidents, surveillance participation, key outcomes); and
- o A case-management / CRM-style layer for outreach, notifications, reminders, and tracking of services and decisions for exposed or high-risk employees

Please enter at most 2000 characters

20. **FedRAMP / ATO Readiness.** State your current FedRAMP authorization status (if applicable): level (Low/Moderate/High), sponsoring agency, and date of authorization.

Please enter at most 2000 characters

21. **FedRAMP / ATO Readiness.** If not currently FedRAMP Moderate (or higher) authorized, describe your plan and estimated timeline to achieve FedRAMP Moderate.

Please enter at most 2000 characters

22. **FedRAMP / ATO Readiness.** Describe any experience obtaining a DHS Authority to Operate (ATO) or other federal ATO. Include approximate timelines and key steps.

Please enter at most 2000 characters

23. **Data Residency, DR/BCP.** Explain how you ensure all DHS data remain within a DHS-controlled cloud environment and within the continental United States. \*

24. **Data Residency, DR/BCP.** Describe your disaster recovery and business continuity capabilities (e.g., RTO, RPO, failover model). \*

Please enter at most 2000 characters

25. **Security and Identity.** Describe support for Personal Identity Verification (PIV) card authentication and integration with federal identity/SSO solutions. \*

Please enter at most 2000 characters

26. **Security and Identity.** Explain how your solution protects PHI and PII (encryption at rest and in transit, key management, audit logging, RBAC, breach notification). \*

Please enter at most 2000 characters

27. **Security and Identity.** Describe how your solution can operate within a DHS-controlled environment (e.g., hosted on DHS infrastructure or within a DHS tenant) while still allowing you to deliver updates, patches, and enhancements. \*

Please enter at most 2000 characters

### Section 3 – Occupational Data Governance, Privacy, and Access Control

28. **Occupational Data Governance.** Explain how your system distinguishes and enforces rules for:
- Pre-employment/pre-placement vs. treatment encounters vs medical surveillance;
  - Administrative/fitness-for-duty exams vs. clinical care;
  - Occupational vs. non-occupational visits (for clinics that may provide both) \*

Please enter at most 2000 characters

29. **Occupational Data Governance.** Describe how you support compliance with OSHA 1910.1020, ADA, GINA, HIPAA, the Privacy Act, and state privacy laws in the context of employee occupational health. \*

Please enter at most 2000 characters

30. **Role-Based Access Control and Segmentation.** Describe your RBAC model, including how it restricts access to occupational health records, segregates specially protected sub-records (e.g., mental/behavioral/EAP data), and supports 'minimum necessary' views for non-clinical roles (e.g., HR, supervisors, safety).

Please enter at most 2000 characters

31. **Role-Based Access Control and Segmentation.** Can DHS administrators configure access rules and "minimum necessary" views without vendor code changes?

32. **Auditing and Retention.** Describe your audit logging capabilities (who accessed what, when, from where, and what actions were taken). Completeness of chart records and documentation.

Please enter at most 2000 characters



33. **Auditing and Retention.** Explain how you support long-term retention and legal holds for occupational records (e.g., 30+ years of retention, export capabilities, record separation at separation/retirement).

Please enter at most 2000 characters

## Section 4 – Occupational Health Functional Capabilities

34. **Encounter Types and Templates.** Briefly describe how your system supports configurable encounter types and templates for the following occupational visit types:

- Pre-employment/pre-placement exams;
- Periodic and fitness-for-duty exams;
- Medical surveillance visits;
- Injury/illness visits;
- Return-to-work/light duty determinations;
- Document-review-only encounters;
- Non-occupational urgent/acute care visits (for clinics that provide such care);
- Mental health/EAP-related occupational encounters.

Please enter at most 2000 characters

35. **Encounter Types and Templates.** Can DHS staff create and modify encounter types, templates, and forms without vendor development?

Please enter at most 2000 characters

36. **Fitness-for-Duty and Medical Standards.** Describe how your solution manages medical standards by position or job type (e.g., law enforcement, firearms-qualified, special duty positions) and supports recording/tracking of fitness-for-duty or clearance decisions over time (including alerts for expiring/overdue exams).

Please enter at most 2000 characters

37. **Pre-Employment Exams and External Exam Ingestion.** Describe how your system supports pre-employment medical exam workflows and integration with applicant tracking or Component HR systems.

Please enter at most 2000 characters

38. **Pre-Employment Exams and External Exam Ingestion.** Explain how external exam results (e.g., from contracted clinics) can be ingested as structured data or documents and transformed into the DHS employee occupational health record without unnecessary repeat exams.

Please enter at most 2000 characters

39. **Workplace Injury/Illness, Exposures, and Return-to-Work.** Describe how your solution documents work-related injuries, illnesses, and exposures, and links them to jobs, locations, exposure programs, and Similar Exposure Groups (SEGs) where applicable.

Please enter at most 2000 characters

40. **Workplace Injury/Illness, Exposures, and Return-to-Work.** Explain how your system generates OSHA 300/300A/301 forms and required reports.

Please enter at most 2000 characters

41. **Drug and Alcohol Testing.** Describe your capabilities for managing federal drug and alcohol testing programs (e.g., compliance with 49 CFR Part 40, Executive Order 12564) and associated Medical Review Officer (MRO) workflows.

Please enter at most 2000 characters

42. **Immunizations and Health Surveillance Programs.** Describe how your system manages immunizations (status tracking, reminders, campaigns) and supports occupational health surveillance programs (e.g., respiratory protection, hearing conservation, TB, bloodborne pathogens, hazard-specific programs such as lead, noise, heat/cold), including how surveillance panels are defined, mapped to employees (job, exposure, location, SEG), and updated over time.

Please enter at most 2000 characters

43. **Medical Readiness Dashboards and Analytics** Describe your dashboard and reporting capabilities for workforce medical readiness and program performance (e.g., injury trends, illness patterns, surveillance compliance) at individual, unit, Component, and enterprise levels.

Please enter at most 2000 characters

44. **Medical Readiness Dashboards and Analytics** Can DHS power users create and modify dashboards and ad-hoc reports without vendor involvement?

Please enter at most 2000 characters

45. **Structured Occupational and Exposure Data (Exposure-Informed Care)** Explain how your system captures occupation, position, location, and exposure information (e.g., job history, tasks, SEGs, known exposure incidents, site assessments).

Please enter at most 2000 characters

46. **Structured Occupational and Exposure Data (Exposure-Informed Care)** Do you support NIOSH ODH, NAICS, SOC, O\*NET, or similar occupational data standards?

Please enter at most 2000 characters

47. **Structured Occupational and Exposure Data (Exposure-Informed Care)** Describe how structured occupational and exposure data are used to support:

- Assignment and management of surveillance programs;
- Exposure-informed clinical decision-making at the point of care (e.g., "Exposure and Occupational Summary" views);
- Cohort/registry management for exposed groups and incidents;
- Analytics and reporting for exposure-related outcomes

Please enter at most 2000 characters

## Section 5 – Interoperability and Integration

48. **Standards-Based Interoperability** Describe your support for HL7 FHIR R4, HL7 v2.x, USCDI, and other relevant interoperability standards.

Please enter at most 2000 characters

49. **Standards-Based Interoperability**

Provide examples of standards-based integrations (e.g., labs, immunization registries, external EHRs) that you have successfully implemented.

Please enter at most 2000 characters

50. **HR and Personnel System Integration** Describe how your solution would integrate with federal and Component HR systems (e.g., NFC, USA Staffing, eOPF, Component HRMS) for:

- Employee demographic data;
- Position and job code information;
- Onboarding and separation workflows.

Please enter at most 2000 characters

51. **HR and Personnel System Integration**

Explain how employee identity, position, and organizational information are kept synchronized across systems, and how this drives enrollment into surveillance/readiness programs.

Please enter at most 2000 characters

52. **Devices and Ancillary Systems** Describe how you integrate with devices and ancillary systems such as audiometers, spirometers, point-of-care test devices, and external laboratory and radiology systems.

Please enter at most 2000 characters

53. **Data Migration** Describe your approach to migrating data from:

- Paper records;
- Scanned PDFs;
- Spreadsheets;
- Legacy occupational health applications

Please enter at most 2000 characters

54. **Data Migration** Discuss lessons learned from similar large migrations and how you minimize disruption and data quality issues (e.g., handling incomplete location or exposure data).

Please enter at most 2000 characters

55. **Analytics and Enterprise Platforms** Describe how your solution supports analytics in a DHS environment, including exporting identified and de-identified data to external analytics platforms and/or providing embedded BI/reporting capabilities.

Please enter at most 2000 characters

## Section 6 – Implementation, Configuration, and Support

56. **Implementation Approach and Timeline** Provide a high-level overview of your recommended implementation approach and indicative timelines (e.g., pilot, major phases, full rollout) for an enterprise of this scale.

Please enter at most 2000 characters

57. **Phasing Strategy** Describe how you would phase implementation across DHS Components and/or functional domains (e.g., direct-care clinics first; exam-review-only Components first; surveillance functions first).

Please enter at most 2000 characters

58. **Phasing Strategy**

Identify dependencies or prerequisites you recommend (e.g., HR integration, identity integration, data cleanup, Component readiness).

Please enter at most 2000 characters

59. **Configuration vs. Customization** Identify which elements can be configured by DHS (e.g., templates, workflows, RBAC, surveillance panels, dashboards) and which would require vendor development.

Please enter at most 2000 characters

60. **Configuration vs. Customization**

Describe how your solution accommodates changes over time (e.g., new surveillance programs, new job standards, new Components) without extensive custom code.

Please enter at most 2000 characters

## Section 7 – Acquisition Strategy, Pricing, and Price Drivers

61. **Contract Vehicles and Procurement Approach** Identify federal contract vehicles through which your solution is currently available (e.g., GSA MAS, NASA SEWP, Alliant, CIO-SP, 8(a) STARS, VETS, agency BPAs). For each, provide contract number and relevant SINS or task areas.

Please enter at most 2000 characters

62. **Contract Vehicles and Procurement Approach**

If your solution is not currently on a government-wide vehicle, what acquisition approach would you recommend (e.g., agency-specific IDIQ, BPA)?

Please enter at most 2000 characters

63. **Contract Vehicles and Procurement Approach**

Describe any use of OTAs or other mechanisms for similar projects.

Please enter at most 2000 characters

64. **Pricing Model and Licensing** Describe at a high level, your standard pricing model.

Please enter at most 2000 characters



65. **Included and Optional Modules** Provide a high-level summary of which occupational health capabilities are typically included in your base product vs. priced as optional modules. At a minimum, address:

- Pre-employment and occupational exams;
- Medical surveillance engine (panels, recalls, longitudinal tracking);
- Injury/illness and return-to-work case management;
- Immunizations;
- Drug and alcohol testing program support;
- Employee/occupational health portal;
- Analytics/BI/reporting;
- Device integration (e.g., audiometer/spirometer interfaces);
- Any other major capabilities that you commonly deploy.

Please enter at most 2000 characters

66. **Included and Optional Modules**

For each capability identified in response to Question #65, indicate whether it is:

- Included in base licensing; or
- Licensed/priced separately.

Please enter at most 2000 characters

67. **Rough Order of Magnitude (ROM) Estimates** If readily available, provide a non-binding ROM, provide a rough order of magnitude (ROM) estimate for a DHS-wide Occupational Health EHR / Management Information System deployment. This ROM is not binding, but should be realistic enough to inform DHS budget planning.

Please enter at most 2000 characters

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